

MedVoice AI

Go-to-Market & Competitive Strategy

Market Research — June 2026

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Abstract

Strategic positioning for MedVoice AI in the medical AI receptionist market. Research sources: LetsAskClaire 2026 State of AI Receptionists, DeepCura rankings, AgentZap pricing guide, Vapi cost analysis. Goal: justify premium pricing through privileges beyond cost savings.

1 Market Context

The medical AI receptionist market in 2026 is consolidating around three architectures:

1. **Reasoning AI platforms** (Hyro, Claire) — enterprise, deep EHR
2. **Virtual receptionist + AI** (Smith.ai, Ruby) — hybrid human backup
3. **Developer platforms** (Vapi, Retell) — build-your-own on API

MedVoice occupies category 1+3: **medical-specialized product built on developer-grade runtime**—faster to ship than pure platform, more control than boxed SMB tools.

2 Competitive Landscape

Competitor	Price/mo	Strength	Weakness
Hyro	\$10K+	Enterprise EHR	Overkill for independents
DeepCura	\$129/provider	Scribe + reception bundle	Not phone-first
My AI Front Desk	\$65–99	Healthcare focus	Limited customization
Smith.ai	\$95+	Human fallback	Per-call overages
Emitrr	\$100–1K	Independent practices	Generic, not medical-deep
Retell/Vapi DIY	Usage-based	Cheap at scale	No product layer
MedVoice	\$1.5–5K target	Medical FSM + your data	Pre-revenue

3 Market Pricing Benchmarks (Medical, 2026)

Per LetsAskClaire research, US medical practices pay:

Practice Profile	Calls/mo	Market Rate/mo
Solo provider	300–600	\$1,200–2,400
2–3 provider primary care	600–1,200	\$2,400–4,200
4–6 provider multi-specialty	1,200–2,400	\$4,200–7,200
8–12 provider group	2,400–4,000	\$7,200–12,000
Multi-location (15+)	4,000–8,000+	\$12,000–25,000+

Human receptionist benchmark: 2 FTE at \$26/hr loaded = **\$108K/year** (\$9K/mo) vs. AI at **\$60K/year** (\$5K/mo average).

4 MedVoice Pricing Strategy

4.1 Cost Floor (do not price below)

Item	1,000 min/mo clinic
Vapi + providers (all-in)	\$180–330
Hosting + monitoring	\$50–100
Support allocation	\$100–200
Floor	\$330–630/mo

Target **3–5× gross margin** on variable costs → charge **\$1,500–2,500/mo** for solo/small clinic entry.

4.2 Recommended Tier Structure

Tier	Price	Included	Target
Starter	\$1,499/mo	800 min, 1 number, calendar	Solo provider
Professional	\$2,999/mo	2,000 min, 2 numbers, SMS	2–4 providers
Group	\$5,999/mo	5,000 min, 5 numbers, analytics	5–10 providers
Enterprise	Custom	Unlimited*, EHR, SLA	Multi-location

Overage: \$1.50/min (high enough to encourage tier upgrades, low enough vs. human cost).

4.3 Strategic Price Entry

- **Launch (months 1–3):** Pilot pricing \$999/mo for first 5 clinics — “Founding Practice” badge, locked rate for 12 months
- **Growth (months 4–8):** Raise to list price; grandfather pilots
- **Scale (month 9+):** Add Enterprise tier; annual prepay discount (15%)

5 Why Clinics Buy Beyond Price

These are your **privileges**—reasons to choose MedVoice at premium vs. \$65/mo generic tools:

5.1 1. Medical-Specific Safety (non-negotiable)

- Deterministic emergency keyword gate (bypasses LLM) — **VERIFIED** in codebase
- Red-flag routing to clinical staff within seconds
- HIPAA BAA chain managed for clinic (one vendor relationship)
- Audit trail for every call turn (compliance-ready)

5.2 2. Appointment Intelligence

- Real-time Google Calendar freebusy (not just Calendly links)
- Structured lead capture: DOB, insurance, reason for visit

- Service-specific discovery flows (dental, urgent, follow-up)
- Future: no-show recovery outbound campaigns

5.3 3. After-Hours Revenue Recovery

- 62% of patient calls occur outside business hours (industry avg)
- Voicemail = lost appointment; MedVoice books at 2am
- Quantify ROI: “3 after-hours bookings/mo $\times$ \$200 visit = \$600 recovered”

5.4 4. Zero Staff Turnover Tax

- Human receptionist: \$5K recruiting + 3-month ramp per hire
- MedVoice: consistent greeting, zero sick days, instant scale

5.5 5. Multilingual Without Interpreter Line

- Spanish (Phase 3) included — no \$0.50/min interpreter fees
- Same medical terminology accuracy across languages

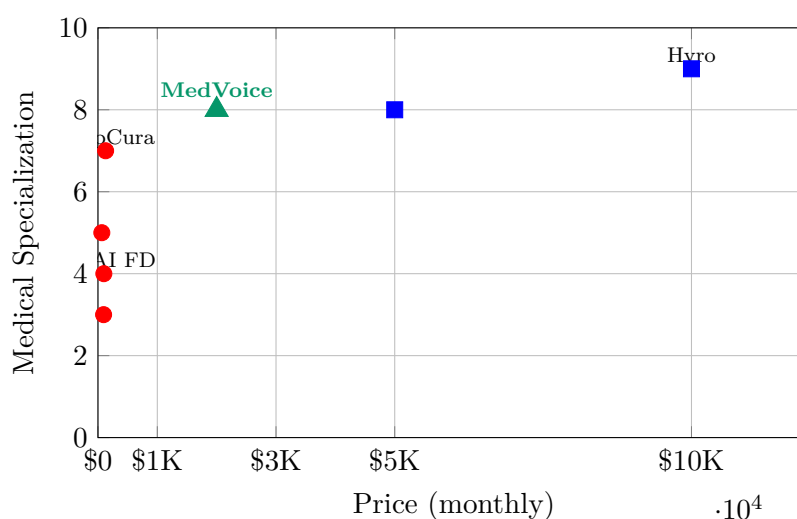
5.6 6. Clinic-Owned Data

- BYOK provider keys — clinic keeps vendor relationships
- Webhook to clinic’s n8n/CRM — data never trapped in platform
- Export all transcripts, leads, recordings on demand

5.7 7. Transparent Dashboard

- Real-time lead feed, emergency alerts, call transcripts
- No black-box AI — see exactly what was said and captured
- Per-service conversion metrics (Phase 4)

6 Competitive Positioning Map



MedVoice targets: **high medical specialization at mid-market price**—above generic SMB tools, below enterprise platforms.

7 Sales Narrative (Elevator to Close)

1. **Problem:** “Your front desk misses 30–40% of after-hours calls. Each missed call is a \$150–300 lost visit.”
2. **Solution:** “MedVoice answers every call in 2 rings, books into your real calendar, and escalates emergencies to your on-call line.”
3. **Proof:** Live demo call to their Google Calendar; show dashboard transcript.
4. **Safety:** “Our emergency detector doesn’t use AI—it’s a hard-coded medical red-flag system.”
5. **ROI:** “At \$2,500/mo you need 13 extra appointments to break even. We typically see 20–40.”
6. **Close:** Founding Practice pilot at \$999/mo, 30-day money-back if <5 bookings.

8 Risk Mitigation for Buyers

Buyer Fear	MedVoice Answer
“AI will say wrong things”	Rule-based emergency gate + human transfer option
“HIPAA violation”	BAA with clinic; BAAs with all providers documented
“Vendor lock-in”	BYOK keys; webhook export; standard APIs
“Hidden per-minute fees”	Flat monthly tier; overage disclosed upfront
“Can’t handle our specialty”	Configurable services, discovery flows, prompts

9 12-Month Revenue Model

Month	Clinics	ARPU	MRR
3	3	\$1,200	\$3,600
6	8	\$2,000	\$16,000
9	15	\$2,500	\$37,500
12	25	\$3,000	\$75,000

At 25 clinics $\times$ \$3K ARPU = **\$900K ARR** by month 12 (aggressive but achievable in underserved independent-practice segment).

10 Key Differentiators to Emphasize in Marketing

1. **Medical FSM, not generic chatbot** — purpose-built conversation states for healthcare
2. **Emergency-first architecture** — safety before convenience
3. **Real calendar booking** — not link-dropping
4. **Your data, your keys** — anti-lock-in positioning
5. **Founding Practice program** — scarcity + early adopter privilege